



GROW LABEL

Revenue Recovery Estimate

A modelled estimate from nine answers

Prepared for	Northgate Dental Group
Prepared	18 September 2026
Basis	Nine answers, modelled. Not a measurement.

This document states every coefficient behind its own figures. Nothing in it has been read from your systems, and no figure in it is a commitment.

THE ESTIMATE

Recoverable revenue, modelled

AT LEAST

\$287,000 a year

Modelled range: up to \$478,000 a year on the same answers.

This figure is a modelled estimate. It was produced by applying a fixed set of coefficients to the nine answers given, and it is not a measurement of your practice: nothing in it has been read from your systems, and no part of it has been observed.

It is stated as "at least" on purpose. The model holds the headline at sixty per cent of what its own arithmetic produces, and rounds down at every stage, because this number exists to be beaten by the assessment that follows rather than to be defended.

The paid revenue assessment is the measurement. It reads a defined window of your own contact and scheduling data and reports what was actually lost, priced from your own fee schedule.

AND THE TIME IT COSTS TODAY

530 front-desk hours a year

Alongside the revenue, an estimate of the front-desk hours a year currently spent returning missed calls, chasing no-shows and working a lapsed list by hand. Owners who argue with a revenue figure rarely argue with the hours.

YOUR ANSWERS

The answers this was built from

Nine inputs, exactly as given. If any line here is wrong the figure is wrong with it, and the fastest way to a better estimate is to correct the line and run it again.

01 What kind of practice do you run?	Dental
02 How many locations?	Four to seven
03 How many active patient or client records across all locations?	4,000–10,000
04 Appointments in a typical week, all locations combined?	240 a week
05 What is a completed appointment worth on average?	\$200–\$400
06 How many people work reception or the front desk, across all locations?	Four to six
07 How are calls handled when nobody can pick up?	It goes to voicemail
08 Roughly what share of appointments are no-shows or late cancellations?	Not sure
09 When did you last run a campaign to bring back patients who had lapsed?	Never

DERIVED FROM THOSE ANSWERS

Trading weeks a year	48
Assumed dormant share	30% of active records, the default for dental practices
No-show rate applied	15%, the default where the answer was "not sure"

Picks up the calls your desk cannot get to.

ESTIMATED A YEAR

\$266,100

THE LEAK

Calls that ring out, sit in a queue or arrive out of hours. A call nobody answers creates no record anywhere, so this is the only loss on the list that never appears in a management report.

WHAT PRODUCED THIS FIGURE

- 28,800 calls a year, at 2.5 for every one of your 11,520 appointments.
- 22% of them assumed to go unanswered, from how your practice handles a call nobody can pick up — about 6,336 calls.
- 35% of those callers were ringing to book, and 40% of those are treated as recoverable, at \$300 an appointment.

WHAT IT WORKS WITH

Your phone system and your appointment diary. It sits behind the number you already publish and books into the diary you already keep; nobody is asked to dial differently.

WHAT YOU WOULD SEE REPORTED

Calls answered that would otherwise have rung out, what each caller wanted, and the appointments booked from them — counted only where the appointment exists in your system.

Answers a new enquiry while the person is still deciding.

ESTIMATED A YEAR

\$33,100

THE LEAK

Web forms, messages and callback requests answered hours after they arrive. The enquiry is not lost, it is answered late, by which point it has usually been answered somewhere else first.

WHAT PRODUCED THIS FIGURE

- 922 web and form enquiries a year, at 8% of your appointment volume.
- A baseline conversion of 20%, improving to 32% once a first response lands inside the window people still book in.
- The difference between those two, at \$300 an appointment.

WHAT IT WORKS WITH

The forms on your website, your shared inbox and your messaging channels. It reads what arrives and responds inside the window in which people still book.

WHAT YOU WOULD SEE REPORTED

Time to first response by channel and by hour, and the enquiries that became appointments — again, only where the appointment exists in your system.

Protects a schedule that is already full.

ESTIMATED A YEAR

\$103,600

THE LEAK

No-shows and late cancellations. The appointment was made, the slot was held, and nothing arrived — so the practice paid for the capacity twice and collected nothing.

WHAT PRODUCED THIS FIGURE

- 1,728 no-shows and late cancellations a year, at the 15% rate assumed for your practice type.
- A 20% reduction in them, set beneath the range this work is usually held to.
- Each recovered slot at \$300.

WHAT IT WORKS WITH

Your appointment diary and your patient contact channels. It works the confirmation, the reminder and the gap that opens when one is declined.

WHAT YOU WOULD SEE REPORTED

No-shows and late cancellations by site, by day and by clinician, and the slots that were refilled after one.

Works the records that stopped moving.

ESTIMATED A YEAR

\$75,600

THE LEAK

Patients and clients who have not been back and are not booked. They are already on your books, already know you, and cost nothing to acquire a second time.

WHAT PRODUCED THIS FIGURE

- 7,000 active records, of which 30% — about 2,100 — are assumed dormant.
- 100% of that list still addressable, from when you last worked it deliberately.
- A 12% return rate, at \$300 an appointment.

WHAT IT WORKS WITH

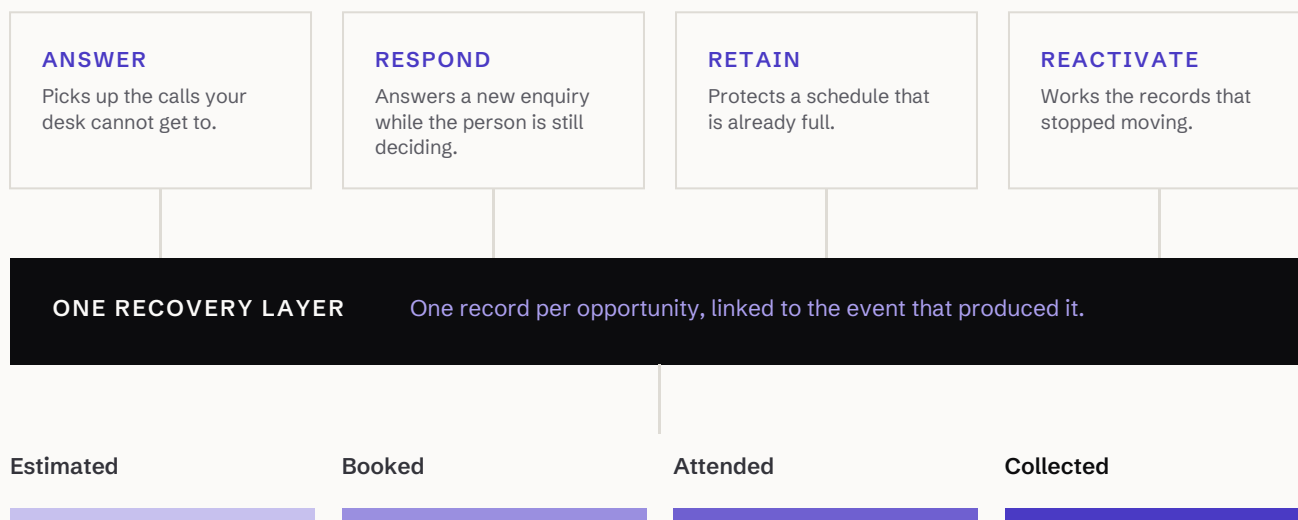
Your records system and your contact channels. It segments the dormant list and works it in batches you approve, at a pace your desk can absorb.

WHAT YOU WOULD SEE REPORTED

The dormant list by how long since last visit, who was contacted, who replied, and who booked.

How the four connect

Each module works a different loss and none of them shares a definition with another, so nothing is counted twice. What they produce lands in one recovery layer, and that layer reports at four separate stages — estimated, booked, attended and collected. Only the last of those is revenue.



A figure is promoted from one stage to the next only on a record in your own system, never on an outcome we inferred.

Only collected value is revenue. The other three are stages on the way to it, and this report never adds them together.

What happens next

The revenue assessment is the paid engagement this estimate points at. It is a measurement exercise, deliberately not a pilot, and it commits neither side to anything beyond it.

What it measures	Where demand was lost across a defined historical window — by channel, by site and by hour — using your definitions, and the value of it priced from your own fee schedule.
What it needs	A read-only export of contact and scheduling data for the window, and your fee schedule. No clinical records, no treatment notes, and nothing is installed or changed in your systems.
How long it takes	A scoping call to agree the window and the definitions, then the written analysis back inside three weeks of the data arriving.
What you keep	The written analysis, whether or not you go further. It is yours, including the parts that say a loss is smaller than this estimate suggested.

THE GUARANTEE

If the assessment does not identify recoverable revenue worth at least what the assessment cost, the fee is returned in full. You keep the written analysis either way. The guarantee covers the assessment fee and nothing beyond it: no result from any module is guaranteed, and no figure in this report is promised.

TO START

Request an assessment at thegrowlabel.com/contact. The first step is a scoping call to agree the window and the definitions; nothing is installed and nothing in your systems changes.

Methodology and limits

Everything below is the whole of the working. Nobody else shows theirs, which is the reason this page exists: a number you can audit is worth more than a number you cannot, even when the auditable one is smaller.

The coefficients are fixed. They are not tuned per practice, they are not drawn from your sector, and they do not adjust to make a result look better. Each one is set at or below the conservative end of the range the operation is usually worked at.

EVERY COEFFICIENT USED

Trading weeks a year	48. Four weeks given back to closures and holiday.
Calls per appointment	2.5. Applied to your appointment volume to reach call volume.
Missed call rate	22%, from how your practice handles a call nobody can pick up. The four rates are 22%, 12%, 20%, 18%.
Booking intent	35% of missed callers were ringing to book rather than to ask.
Recoverable share	40% of those are treated as recoverable. Held well under half deliberately.
Enquiry rate	8% of appointment volume arrives as a web or form enquiry.
Enquiry conversion	20% assumed today, 32% after a fast first response. The improved figure is capped below best case, and response time is not asked for — it would be a tenth question.
No-show rate	15%. Default for dental practices, used because the answer was "not sure".
No-show reduction	20% of them prevented. Set under the range this work is usually held to.
Dormant share	30% of active records, the default for dental practices.
Campaign factor	100%, from when you last worked the lapsed list. The four factors are 100%, 85%, 55%, 30%.
Reactivation rate	12% of the addressable dormant list returns. Set beneath the range this is usually worked at.
Appointment value	\$300. The midpoint of the band you chose, or its lower bound where the band is open-topped.
Active records	7,000. Same rule: the midpoint of the band, or its lower bound where it is open-topped.
Headline share	60% of the modelled total. The headline is deliberately the low end of the range, and every figure is rounded down.

Front-desk minutes

3 minutes per missed call returned, 5 per no-show chased, 2 per dormant record worked by hand.

WHERE THE RANGES COME FROM

Each coefficient above is set at or beneath the conservative end of the range the underlying operation is commonly worked at, and several are set well beneath it. Where a range is wide, the model takes the bottom of it. Where an input was answered "not sure", the model takes the default for the practice type rather than the most favourable option.

WHAT THIS ESTIMATE IS NOT

- It is not a measurement. No part of it has been read from your systems.
- It is not a forecast. It describes the size of an opportunity, not the likelihood of collecting it.
- It is not revenue. Recovered value becomes revenue only once it is collected, and it is gross rather than margin.
- It is not a benchmark, and it is not a comparison against other practices. No published average describes a specific practice.
- It is not a commitment. Actual recoverable revenue varies by practice, and no figure in this document is promised.